BASELINE → TREATMENT → FOLLOW-UP

Checks across your treatment

This is a relative treatment map, not a booking calendar. Ask who books each test and reviews the result. Extra tests may be needed for symptoms.

Check	Before starting	First infusion	Between infusions	Course review	After treatment
Blood count, kidney, liver and glucose	Baseline blood tests	Results reviewed before dosing	Before every 3-week cycle	Continue checks during holds/ restarts as directed	Follow unresolved abnormalities
Thyroid and other hormones	Baseline thyroid function; other hormones if needed	Review symptoms and hormone medicines	Thyroid every 3–6 weeks; cortisol if clinically indicated	Confirm replacement and steroid-taper plan	Continue hormone follow-up if needed
Symptoms and immune effects	Discuss existing illnesses and warning symptoms	Tell the nurse immediately about a reaction	Report new symptoms on any day	Hold/restart decisions need clinical review	Late immune effects remain possible
Cancer response and course length	Confirm the exact lung setting	Agree how benefit will be assessed	Scans on your consultant’s schedule	Adjuvant up to 12 months; advanced/maintenance individual review	Agree surveillance or next treatment

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00544 v14 · official regimen

THE WHOLE TREATMENT JOURNEY

Atezolizumab 1200 mg · every 3 weeks · lung cancer: treatment and checks

Lung settings: previously treated NSCLC; first-line PD-L1-high metastatic NSCLC; adjuvant NSCLC after surgery and platinum chemotherapy; ES-SCLC maintenance after induction.

Atezolizumab 1200 mg is given by intravenous infusion on Day 1 every 3 weeks (21 days). The first infusion lasts about 60 minutes; later infusions may last 30 minutes if tolerated. The visit includes checks and preparation. This protocol also offers a separately prescribed 1875 mg injection under the skin of the thigh over about 7 minutes every 3 weeks. The IV and under-skin doses are not interchangeable milligram for milligram. Adjuvant: maximum 12 months. Advanced NSCLC and ES-SCLC maintenance: until progression or unacceptable toxicity. ES-SCLC requires completed induction; this suite contains no induction chemotherapy.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First infusion	Between infusions	Course review	After treatment
Treatment NCCP 00544	Confirm exact indication	1200 mg IV; first 60 minutes	Day 1 every 3 weeks	Adjuvant maximum 12 months; advanced/maintenance review	Follow-up and late immune effects
Blood count, kidney, liver and glucose NCCP 00544	Baseline blood tests	Results reviewed before dosing	Before every 3-week cycle	Continue checks during holds/ restarts as directed	Follow unresolved abnormalities
Thyroid and other hormones NCCP 00544 · SMPC	Baseline thyroid function; other hormones if needed	Review symptoms and hormone medicines	Thyroid every 3–6 weeks; cortisol if clinically indicated	Confirm replacement and steroid-taper plan	Continue hormone follow-up if needed
Symptoms and immune effects NCCP 00544 · SMPC	Discuss existing illnesses and warning symptoms	Tell the nurse immediately about a reaction	Report new symptoms on any day	Hold/restart decisions need clinical review	Late immune effects remain possible
Cancer response and course length NCCP 00544	Confirm the exact lung setting	Agree how benefit will be assessed	Scans on your consultant’s schedule	Adjuvant up to 12 months; advanced/maintenance individual review	Agree surveillance or next treatment

What each check is for

1 Before first dose

Before starting

Verify setting and eligibility

Ask / act: Confirm biomarkers, baseline tests, consent, prescription and alert card.

Sources: NCCP 00544

2 Course review

During treatment

Duration differs by setting

Ask / act: Document continuation, hold or stop and arrange follow-up.

Sources: NCCP 00544

3 After treatment

After the last dose

Immune effects may be delayed

Ask / act: Provide urgent contact and follow-up plan.

Sources: NCCP 00544 · SMPC

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-29 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.26